INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Pembrolizumab 400 mg + carboplatin AUC 5 / paclitaxel → dose-dense AC

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**21 days**
Intervals differ by treatment phase

**Treatment stages**
**4**
Explained in the treatment reference

**Regimen source**
**NCCP 00860**
Version 2

## Treatment stages

01 **Phase 1 · carboplatin and paclitaxel**
About 12 weeks

02 **Phase 2 · dose-dense AC**
About 8 weeks

03 **Surgery and pathology**
After recovery

04 **After surgery · pembrolizumab alone**
Separate phase

Your team confirms the active stage and dates. The full treatment guide explains each step.

## Inside your handbook


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**My name** _____

**Hospital / oncology team** _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

# Understanding your treatment

Pembrolizumab 400 mg + carboplatin AUC 5 / paclitaxel → dose-dense AC · National Cancer Control Programme (NCCP) 00860

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathlessness, chest symptoms, severe diarrhoea, inability to drink, jaundice, a painful swollen leg or a severe rash promptly. You do not need to identify which medicine caused the symptom.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

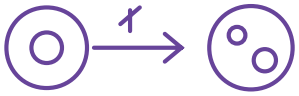
## Medicines in the active phase

Treat the cancer before surgery as part of a plan intended to reduce the risk of it returning.

IMMUNOTHERAPY

## Pembrolizumab

Keytruda



### Releases an immune brake

PD-1 is a control switch on T cells, a type of immune cell. When it connects with PD-L1 or PD-L2, the T cell receives a signal to reduce its activity. Some cancers use this pathway to escape immune attack. Pembrolizumab is an antibody that blocks PD-1, helping T cells recognise and attack the cancer. It does not directly poison cancer cells like chemotherapy. Releasing this brake can also allow immune cells to attack healthy organs, which explains the different side-effect pattern and why symptoms can appear after treatment stops.

Immune cell → Brake blocked → Cancer cell

### What to watch for

The immune system can also inflame healthy organs. New cough, diarrhoea, unusual weakness, severe headache, thirst or a spreading rash need prompt advice. Effects can start during treatment or after it finishes.

PLATINUM CHEMOTHERAPY

## Carboplatin



### Damages deoxyribonucleic acid (the cell's genetic material) (DNA) inside cells

Carboplatin creates links in DNA, making it harder for cells to copy and repair their genetic instructions. Its dose depends on kidney function as well as the intended regimen.

Platinum enters cell → DNA links form → Copying is disrupted

### What to watch for

Keep blood-test appointments. Fever, unusual bleeding, repeated vomiting or infusion symptoms need prompt advice.

TAXANE CHEMOTHERAPY

## Paclitaxel

Taxol (conventional paclitaxel)



### Locks the cell division machinery

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

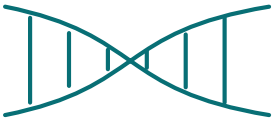
Microtubules form → Structures remain locked → Division is interrupted

### What to watch for

Report tingling or numbness before the next weekly dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

ANTHRACYCLINE CHEMOTHERAPY

## Doxorubicin



### Interferes with DNA handling

Doxorubicin interferes with enzymes and DNA processes needed for cell division. It can also affect heart muscle, so the team considers previous exposure and cardiac risk.

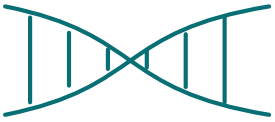
DNA handling disrupted → Cell damage → Division reduced

### What to watch for

Report pain or burning at the infusion site immediately. New breathlessness or chest symptoms need prompt review. Red urine for a short time can be a drug-colour effect; pain, clots or persistent bleeding still need advice.

ALKYLATING CHEMOTHERAPY

### Cyclophosphamide



**Damages DNA needed for growth**

Cyclophosphamide is activated in the body and damages DNA. This helps stop dividing cancer cells but can also affect blood-cell production and reproductive tissues.

Drug activated → DNA damage → Cell division affected

**What to watch for**

Use the prescribed anti-sickness and hydration plan. Report fever, painful urination or blood in the urine.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## Why might my team recommend this treatment?

This treatment is used for eligible high-risk triple-negative breast cancer before surgery, then continued with immunotherapy after surgery when appropriate. Chemotherapy damages cancer cells, while pembrolizumab helps immune cells recognise and attack them. The team considers cancer extent, fitness, heart health and whether immunotherapy is suitable.

### Why more than one phase?

Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 5 on Day 1. Pembrolizumab 400 mg on Day 1 of cycles 1 and 3 only (every six weeks). Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 5 and 8 only; the six-week clock continues across the change in chemotherapy. granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) support is required for every dose-dense AC cycle; the team prescribes its product and timing separately.

### What determines the next step?

After surgery: pembrolizumab 400 mg every 42 days for five doses, beginning six weeks after cycle 8 Day 1, subject to recovery, surgery and the oncology plan. Progression preventing definitive surgery or unacceptable pembrolizumab toxicity changes this plan; postoperative treatment is not automatic.

SACTCHECK · CHAPTER 02

# Your schedule, tests and appointments

Pembrolizumab 400 mg + carboplatin AUC 5 / paclitaxel → dose-dense AC · NCCP 00860

## How the phases connect

### Treatment stages

01 **Phase 1 · carboplatin and paclitaxel**
About 12 weeks

02 **Phase 2 · dose-dense AC**
About 8 weeks

03 **Surgery and pathology**
After recovery

04 **After surgery · pembrolizumab alone**
Separate phase

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ABOUT 12 WEEKS

### Phase 1 · carboplatin and paclitaxel

**Four 21-day cycles**

Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 5 on Day 1. Pembrolizumab 400 mg on Day 1 of cycles 1 and 3 only (every six weeks).

ABOUT 8 WEEKS

### Phase 2 · dose-dense AC

**Four 14-day cycles**

Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 5 and 8 only; the six-week clock continues across the change in chemotherapy. granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) support is required for every dose-dense AC cycle; the team prescribes its product and timing separately.

AFTER RECOVERY

### Surgery and pathology

**Individual timing**

The operation is planned after recovery; the pathology result helps decide the next treatment.

SEPARATE PHASE

### After surgery · pembrolizumab alone

**Confirm the prescribed schedule**

After surgery: pembrolizumab 400 mg every 42 days for five doses, beginning six weeks after cycle 8 Day 1, subject to recovery, surgery and the oncology plan. Progression preventing definitive surgery or unacceptable pembrolizumab toxicity changes this plan; postoperative treatment is not automatic.

The phases run in order. The columns show sequence, not equal time intervals. No dates or other information need to be entered.

BASELINE → TREATMENT → FOLLOW-UP

## When checks happen

Relative sequence, not booked dates or equal time intervals. Baseline electrocardiogram (ECG) is distinct from risk-based echo. No routine three-monthly computed tomography (CT) or echo is implied. The team books and reviews results.

| Check | Before starting | Phase 1 · weeks 1–12 | Before / during AC | Surgery / postoperative | After treatment |
|---|---|---|---|---|---|
| **Blood tests** | full blood count (FBC), renal and liver; verify GFR | Weekly before planned doses | Before each 14-day AC cycle | Before each pembrolizumab cycle | Symptoms or abnormalities determine follow-up |
| **ECG / echo** | ECG and cardiac risk review; echo if indicated | Symptoms prompt earlier assessment | Review before first doxorubicin; further echo by risk / symptoms | Review any cardiac symptoms or previous abnormalities | Risk-based cardiac follow-up |
| **Cancer imaging** | Breast/nodal imaging; staging as indicated | Clinical response review; image for concern | Assess response and plan surgery | Pathology review; agree next steps | Breast surveillance; no routine CT/ markers when asymptomatic |
| **Thyroid / hormones** | Thyroid baseline; hepatitis screening per source | Thyroid every 3–6 weeks | Continue immune monitoring | Continue while on pembrolizumab | Delayed immune symptoms still require review |

| Check | Before starting | Phase 1 · weeks 1–12 | Before / during AC | Surgery / postoperative | After treatment |
|---|---|---|---|---|---|
| **Surgery / radiation** | Agree overall multidisciplinary team (MDT) pathway | Systemic treatment first | Complete neoadjuvant phase if appropriate | Surgery → pathology → radiation if indicated | Follow-up and late-effect support |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00860 v2 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · ASCO · neoadjuvant breast therapy and response assessment

THE WHOLE TREATMENT JOURNEY

# Pembrolizumab 400 mg + carboplatin AUC 5 / paclitaxel → dose-dense AC: the whole pathway

### Triple-negative breast cancer · neoadjuvant and adjuvant immunotherapy

Phase 1: four 21-day cycles; paclitaxel on Days 1, 8 and 15, carboplatin on Day 1. Phase 2: four AC cycles every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 1, 3, 5 and 8; after surgery, every 42 days for five doses. See the whole-treatment map for monitoring and sequencing.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Phase 1 · weeks 1–12 | Before / during AC | Surgery / postoperative | After treatment |
|---|---|---|---|---|---|
| **Cancer medicines** NCCP 00860 | Confirm diagnosis and suitability | Four 21-day cycles: weekly paclitaxel; carboplatin AUC 5 Day 1. Pembrolizumab C1 and C3 Day 1. | Four AC cycles every 14 days. G-CSF each cycle. Pembrolizumab C5 and C8 Day 1. | Surgery → pathology; pembrolizumab 400 mg every 42 days ×5 if appropriate | Review late effects and follow-up |
| **Surgery** EARLY · NEO | Discuss operation options | After systemic treatment | Plan after recovery | Lumpectomy or mastectomy as agreed | Surgical follow-up |
| **Radiotherapy** EARLY | Assess indication | Not a routine concurrent chemotherapy phase | Plan with surgery / radiation teams | After surgery if indicated; may overlap pembrolizumab if agreed | Late-effect review |
| **Blood tests** NCCP 00860 | FBC, renal and liver; verify GFR | Weekly before planned doses | Before each 14-day AC cycle | Before each pembrolizumab cycle | Symptoms or abnormalities determine follow-up |

| Care / check | Before starting | Phase 1 · weeks 1–12 | Before / during AC | Surgery / postoperative | After treatment |
|---|---|---|---|---|---|
| **ECG / echo**<br>NCCP 00860 · ESC | ECG and cardiac risk review; echo if indicated | Symptoms prompt earlier assessment | Review before first doxorubicin; further echo by risk / symptoms | Review any cardiac symptoms or previous abnormalities | Risk-based cardiac follow-up |
| **Imaging / markers**<br>EARLY · NEO | Breast/nodal imaging; staging as indicated | Clinical response review; image for concern | Assess response and plan surgery | Pathology review; agree next steps | Breast surveillance; no routine CT/ markers when asymptomatic |
| **Thyroid / hormones**<br>NCCP 00860 | Thyroid baseline; hepatitis screening per source | Thyroid every 3–6 weeks | Continue immune monitoring | Continue while on pembrolizumab | Delayed immune symptoms still require review |

## What each check is for

1

### Blood tests

FBC, renal and liver tests weekly during the carboplatin/paclitaxel phase, then before every AC cycle. Continue the required blood tests before each postoperative pembrolizumab cycle.

Check safety before the planned dose.

**Ask / act:** Agree the booking owner, result reviewer and plan if delayed.

2

### ECG / cardiac review

Baseline ECG and cardiac history/risk assessment. NCCP lists baseline echo/multigated acquisition (heart pumping) scan (MUGA) for age over 65. Recheck the cardiac assessment before starting doxorubicin; echo/MUGA during treatment is driven by symptoms, risk or the agreed cardio-oncology plan. There is no automatic three-monthly echo schedule for every person on this pathway.

Check risk before and during anthracycline exposure.

**Ask / act:** Agree the booking owner, result reviewer and plan if delayed.

3

### Cancer imaging

Record baseline breast/nodal findings and stage with imaging appropriate to the presentation. Assess clinical response during neoadjuvant treatment and use the appropriate breast imaging before surgery or sooner for concern. There is no universal three-monthly CT schedule for this early-breast pathway. After local treatment, arrange the agreed breast surveillance; routine CT/positron emission tomography (PET) and tumour markers are not asymptomatic recurrence screening.

Assess response and plan local treatment.

**Ask** / **act:** Agree the booking owner, result reviewer and plan if delayed.

**At the next review, ask**

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Your practical treatment plan

Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 5 on Day 1. Pembrolizumab 400 mg on Day 1 of cycles 1 and 3 only (every six weeks). Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 5 and 8 only; the six-week clock continues across the change in chemotherapy. G-CSF support is required for every dose-dense AC cycle; the team prescribes its product and timing separately. After surgery: pembrolizumab 400 mg every 42 days for five doses, beginning six weeks after cycle 8 Day 1, subject to recovery, surgery and the oncology plan. Progression preventing definitive surgery or unacceptable pembrolizumab toxicity changes this plan; postoperative treatment is not automatic.

STEP 1

### Before the first visit

Blood tests, ECG and cardiac review, medicines review, fertility discussion where relevant and the consent conversation come first. Include the prescribed immune baseline checks.

STEP 2

### Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

### Infusion day

Trained staff check and access the line, give the prescribed premedicines and infusions, and monitor for reactions. Tell the nurse immediately about pain, burning, rash or breathlessness.

STEP 4

### Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

#### Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

#### PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

#### Visit time and transport

Visits include assessment, pharmacy preparation, premedicines, infusions and observation. The first visit and a change in phase can take longer. Ask about transport and any escort needed after sedation or line insertion.

## Tests and checks through treatment

Ask who books each test, who reviews the result, and when you should hear the plan.

### Blood tests

FBC, renal and liver tests weekly during the carboplatin/paclitaxel phase, then before every AC cycle. Continue the required blood tests before each postoperative pembrolizumab cycle.

### ECG and heart ultrasound

Baseline ECG and cardiac history/risk assessment. NCCP lists baseline echo/MUGA for age over 65. Recheck the cardiac assessment before starting doxorubicin; echo/MUGA during treatment is driven by symptoms, risk or the agreed cardio-oncology plan. There is no automatic three-monthly echo schedule for every person on this pathway.

### Thyroid and hormone checks

Baseline thyroid tests; thyroid monitoring every 3–6 weeks during pembrolizumab. Cortisol is checked when clinically indicated; tell the team about recent steroid use when results are interpreted. Immune side effects may appear after treatment ends.

### Scans and tumour markers

Record baseline breast/nodal findings and stage with imaging appropriate to the presentation. Assess clinical response during neoadjuvant treatment and use the appropriate breast imaging before surgery or sooner for concern. There is no universal three-monthly CT schedule for this early-breast pathway. After local treatment, arrange the agreed breast surveillance; routine CT/PET and tumour markers are not asymptomatic recurrence screening.

## Surgery, radiotherapy and follow-up

Other treatments depend on the complete diagnosis and pathology.

### How the treatments fit

Chemotherapy comes before surgery. The surgeon discusses lumpectomy or mastectomy as alternatives according to your situation. Radiotherapy, if indicated, follows surgery and recovery; timing is agreed by both teams. It is not automatically given at the same time as these chemotherapy phases. Radiation may overlap postoperative pembrolizumab if both teams agree; this is not an automatic instruction.

### Hormone tablets and bone scans

This is a triple-negative pathway: endocrine therapy and endocrine-related dual-energy X-ray absorptiometry (bone density) (DEXA) are not automatically part of it. Bone-density testing may still be appropriate for other personal risk factors.

### If symptoms or tests change

The team may pause, change or stop a medicine, reassess imaging or revise the surgical plan. Report concerns promptly.

# Symptoms and supportive care

Pembrolizumab 400 mg + carboplatin AUC 5 / paclitaxel → dose-dense AC · NCCP 00860

## Phase 1 · four 21-day cycles

### Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

| Treatment or check | Timing in this view |
|---|---|
| **Carboplatin + paclitaxel · Day 1** |  **Day 1** intravenous (IV) treatment |
| **Paclitaxel · Day 8** |  **Day 8** IV treatment |
| **Paclitaxel · Day 15** |  **Day 15** IV treatment |
| **Pembrolizumab · C1 and C3 Day 1 only** |  **Day 1** Separate immune schedule |
| **Blood counts / infection** |  **Days 1–21** Fever or sudden illness: call |
| **Nerves / nails** |  **Days 1–21** Report before each weekly visit |
| **Immune symptoms** |  **Days 1–21** Can occur on any day or after treatment |

**The first phase:** Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 5 on Day 1. Pembrolizumab 400 mg on Day 1 of cycles 1 and 3 only (every six weeks).

**Changing to AC:** Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 5 and 8 only; the six-week clock continues across the change in chemotherapy. G-CSF support is required for every dose-dense AC cycle; the team prescribes its product and timing separately.

**Surgery and afterwards:** After surgery: pembrolizumab 400 mg every 42 days for five doses, beginning six weeks after cycle 8 Day 1, subject to recovery, surgery and the oncology plan. Progression preventing definitive surgery or unacceptable pembrolizumab toxicity changes this plan; postoperative treatment is not automatic.

> **Why these bands stay broad:** Treatment markers show the planned days. Symptom bands are broad possibilities, not measured risk curves; serious symptoms can occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00860 v2 · official regimen ↗

> **There are no guaranteed "safe days".** Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override the timeline or diary.

## Phase 2 · four 14-day cycles

### Day 1 → Day 14: a general pattern

| Treatment or check | Timing in this view |
|---|---|
| **AC · Day 1** |  **Day 1** IV treatment |
| **Nausea / appetite** |  **Days 1–5** Early or delayed effects |
| **Blood counts / infection** |  **Days 1–14** Fever or sudden illness: call |
| **G-CSF after chemotherapy** |  **Days 2–14** Exact days follow the prescription |
| **Pembrolizumab · C5 and C8 Day 1 only** |  **Day 1** Separate immune schedule |

| Treatment or check | Timing in this view |
| --- | --- |
| **Immune symptoms** | 1 14<br>**Days 1–14**<br>Can occur on any day or after treatment |

**The first phase:** Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 5 on Day 1. Pembrolizumab 400 mg on Day 1 of cycles 1 and 3 only (every six weeks).

**Changing to AC:** Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 5 and 8 only; the six-week clock continues across the change in chemotherapy. G-CSF support is required for every dose-dense AC cycle; the team prescribes its product and timing separately.

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# Postoperative · 42-day schedule

## Day 1 → Day 42: a general pattern

| Treatment or check | Timing in this view |
| --- | --- |
| **Pembrolizumab · Day 1** | 1 42<br>**Day 1**<br>IV treatment |
| **Immune symptoms** | 1 42<br>**Days 1–42**<br>Report during and after treatment |

**The first phase:** Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 5 on Day 1. Pembrolizumab 400 mg on Day 1 of cycles 1 and 3 only (every six weeks).

**Changing to AC:** Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 5 and 8 only; the six-week clock continues across the change in chemotherapy. G-CSF support is required for every dose-dense AC cycle; the team prescribes its product and timing separately.

**Surgery and afterwards:** After surgery: pembrolizumab 400 mg every 42 days for five doses, beginning six weeks after cycle 8 Day 1, subject to recovery, surgery and the oncology plan. Progression preventing definitive surgery or unacceptable pembrolizumab toxicity changes this plan; postoperative treatment is not automatic.

> **Why these bands stay broad:** Treatment markers show the planned days. Symptom bands are broad possibilities, not measured risk curves; serious symptoms can occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00860 v2 · official regimen ↗

### Between-treatment notes

### Day 1–21: Follow the current phase

Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 5 on Day 1. Pembrolizumab 400 mg on Day 1 of cycles 1 and 3 only (every six weeks). Your team books actual appointments and reviews results. Call about urgent symptoms on any day.

## Support around the medicines

### 1 Separate anti-sickness plans

The platinum/taxane phase and AC need separate prescriptions. Carboplatin Day 1 uses a high-risk plan; paclitaxel-only Days 8 and 15 use a separate low-risk plan. AC uses its own high-risk plan.

### 2 Growth-factor support

G-CSF is prescribed for every dose-dense AC cycle. Ask for the exact injection days and instructions.

### 3 Checks before the next dose

FBC, renal and liver tests weekly during the carboplatin/paclitaxel phase, then before every AC cycle. Continue the required blood tests before each postoperative pembrolizumab cycle. Keep the thyroid and other immune-monitoring appointments.

## Questions worth asking

### When should I call the oncology team?

Call immediately for fever, shivers, sudden illness, new breathing difficulty or concerning symptoms. Do not wait for a scheduled blood test or appointment.

### Why is my chemotherapy changing halfway through?

The planned course uses a platinum/taxane phase followed by AC, which works in a different way. The change is part of the pathway and does not by itself mean the first medicines failed.

### Will I need to enter dates here?

No. The maps explain the sequence. Your clinical team provides actual appointment dates.

### When are ECGs, echoes and blood tests done?

Baseline ECG and cardiac history/risk assessment. NCCP lists baseline echo/MUGA for age over 65. Recheck the cardiac assessment before starting doxorubicin; echo/MUGA during treatment is driven by symptoms, risk or the agreed cardio-oncology plan. There is no automatic three-monthly echo schedule for every person on this pathway. FBC, renal and liver tests weekly during the carboplatin/paclitaxel phase, then before every AC cycle. Continue the required blood tests before each postoperative pembrolizumab cycle.

### Does pembrolizumab follow every chemotherapy visit?

No. Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 5 on Day 1. Pembrolizumab 400 mg on Day 1 of cycles 1 and 3 only (every six weeks). Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 5 and 8 only; the six-week clock continues across the change in chemotherapy. G-CSF support is required for every dose-dense AC cycle; the team prescribes its product and timing separately. After surgery: pembrolizumab 400 mg every 42 days for five doses, beginning six weeks after cycle 8 Day 1, subject to recovery, surgery and the oncology plan. Progression preventing definitive surgery or unacceptable pembrolizumab toxicity changes this plan; postoperative treatment is not automatic.

> **Try explaining the plan back:** Which phase are you in, which medicines are active, and what checks are due before the next phase?

### Pembrolizumab

- Immune inflammation of bowel, lungs, liver, kidneys or skin
- Hormone-gland problems, sometimes permanent
- Rare serious heart or neurological inflammation
- Infusion reaction
- Effects can first appear after treatment ends

### Carboplatin

- Low blood counts, infection and bleeding
- Nausea, vomiting and fatigue
- Allergic reaction, sometimes after previous exposure
- Kidney or electrolyte changes

### Paclitaxel

- Low blood counts and infection
- Numbness / tingling, sometimes lasting
- Hair loss, muscle or joint pain
- Mouth, bowel, skin and nail effects

### Doxorubicin

- Low counts, infection and mouth ulcers
- Nausea, hair loss and fatigue
- Heart damage, sometimes delayed
- Severe tissue injury if the infusion leaks
- Rare later blood cancers

### Cyclophosphamide

- Low counts and infection
- Nausea, vomiting and hair loss
- Bladder irritation / bleeding
- Fertility effects and menopause

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Phase 1 · Day 1 carboplatin combination

**Active medicines:** Carboplatin + Paclitaxel + Pembrolizumab on its separate schedule

**Emetogenic risk:** High · carboplatin AUC ≥4

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally | Day 1 |
| Ondansetron | 16 mg orally | Day 1<br>Individual dose / QT / hepatic review |
| Dexamethasone | 12 mg orally | Day 1<br>Reconcile chemotherapy premedication steroids |
| Aprepitant | 80 mg orally daily | Days 2–3 |
| Metoclopramide | 10 mg orally up to three times daily | Subsequent days: duration and use to be confirmed<br>Individual contraindications, renal function and duration review |

| Medicine | Dose / route | When |
|---|---|---|
| Dexamethasone | 8 mg orally daily | Days 2–4<br>Carboplatin AUC ≥4 antiemetic row; reconcile total steroid exposure |

### Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment.
- Day 8 and later: no routine dexamethasone unless the consultant specifies it for reaction/risk; chlorphenamine continues and any famotidine omission requires consultant/local-policy agreement.
- Reconcile antiemetic and taxane dexamethasone into one prescription; never add duplicate steroid courses.
- Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 5 on Day 1. Pembrolizumab 400 mg on Day 1 of cycles 1 and 3 only (every six weeks).

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

## Phase 1 · Days 8 and 15 paclitaxel

**Active medicines:** Paclitaxel

**Emetogenic risk:** Low

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 8 mg orally | On treatment day<br>Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed |
| Metoclopramide | 10 mg orally up to three times daily as needed | Subsequent days: duration to be confirmed<br>Assess contraindications, renal function and interactions |

### Additional phase-specific support

- Day 8 and later: no routine dexamethasone unless the consultant specifies it for reaction/risk; chlorphenamine continues and any famotidine omission requires consultant/local-policy agreement.
- Reconcile antiemetic and taxane dexamethasone into one prescription; never add duplicate steroid courses.
- No carboplatin on these days; do not repeat the Day 1 delayed antiemetic course automatically.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

## Phase 2 · dose-dense AC

**Active medicines:** Doxorubicin + Cyclophosphamide + Pembrolizumab on its separate schedule

**Emetogenic risk:** High · AC

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally | Day 1 |
| Ondansetron | 16 mg orally | Day 1<br>Individual dose / QT / hepatic review |
| Dexamethasone | 12 mg orally | Day 1<br>No routine Days 2–4 dexamethasone in this NCCP AC antiemetic row |
| Olanzapine | 5 mg orally | Days 1–4<br>Review sedation, falls, interactions and glucose |
| Aprepitant | 80 mg orally daily | Days 2–3 |
| Metoclopramide | 10 mg orally up to three times daily | Subsequent days: duration and use to be confirmed<br>Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications |

### Additional phase-specific support

- Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 5 and 8 only; the six-week clock continues across the change in chemotherapy. granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) support is required for every dose-dense AC cycle; the team prescribes its product and timing separately.
- G-CSF every dose-dense AC cycle is required. Product, start day and duration must be prescribed using the local supportive-care policy; do not administer simultaneously with cytotoxic chemotherapy.
- Confirm cumulative doxorubicin exposure, cardiac assessment and venous-access plan.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

## After surgery · pembrolizumab alone

**Active medicines:** Pembrolizumab

**Emetogenic risk:** Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

### Additional phase-specific support

- After surgery: pembrolizumab 400 mg every 42 days for five doses, beginning six weeks after cycle 8 Day 1, subject to recovery, surgery and the oncology plan. Progression preventing definitive surgery or unacceptable pembrolizumab toxicity changes this plan; postoperative treatment is not automatic.
- No routine cytotoxic antiemetics, taxane premedicines or chemotherapy granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) are carried forward.
- Continue the immune-toxicity and required blood/hormone monitoring.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00860 · VERSION 2

# Pembrolizumab 400 mg + carboplatin AUC 5 / paclitaxel → dose-dense AC

Phase 1: four 21-day cycles; paclitaxel on Days 1, 8 and 15, carboplatin on Day 1. Phase 2: four AC cycles every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 1, 3, 5 and 8; after surgery, every 42 days for five doses. See the whole-treatment map for monitoring and sequencing.

Source reconciled; independent oncology, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

## A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

### 1 Hair / scalp

**Paclitaxel**

Hair loss is common; ask about support and scalp cooling where suitable.

### 2 Nerves / nails

**Paclitaxel**

Report numbness, tingling or painful nails before the next dose.

### 3 Blood / infection

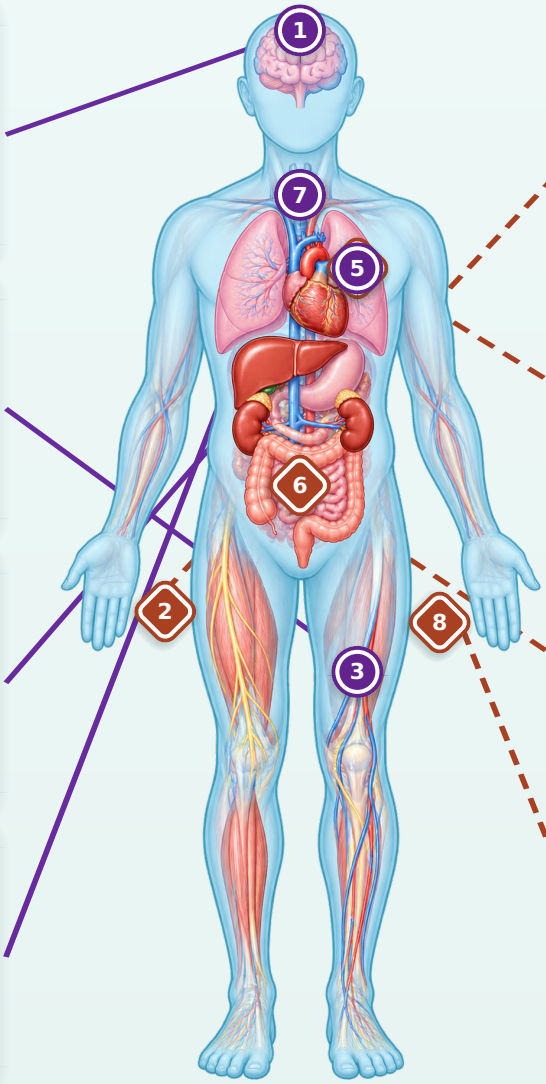
**Carboplatin and paclitaxel**

Fever, shivers, unusual bleeding or sudden illness: call immediately.

### 4 Infusion reaction

**Carboplatin or paclitaxel**

Tell staff immediately about flushing, chest tightness or breathing difficulty.




### 5 Breathing

**Pembrolizumab**

New cough or breathlessness needs prompt assessment.

### 6 Bowel

**Pembrolizumab**

Report diarrhoea early, especially with pain, bleeding or poor intake.

### 7 Hormone glands / energy

**Pembrolizumab**

Report marked weakness, dizziness, headache or unusual thirst.

### 8 Skin / liver symptoms

**Pembrolizumab**

A severe rash, blistering or jaundice needs prompt advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

This printed map shows phase 1. See the next page for all medicines and the online phase-specific maps. Report persistent effects after a phase ends.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

# Your record and contact card

Pembrolizumab 400 mg + carboplatin AUC 5 / paclitaxel → dose-dense AC · National Cancer Control Programme (NCCP) 00860

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

**Phase 1 · reusable record** · copy this sheet for another cycle of the same phase.

## Pembrolizumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |

## Carboplatin · Platinum chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Bleeding / bruising** | □ None | □ New small bruise / brief bleed: report | □ Repeated bleeding: call today | □ Bleeding that will not stop: urgent help | □ Heavy bleeding / collapse: 112/999 |

## Paclitaxel · Taxane chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Numbness / tingling** | □ None | □ Mild change; usual tasks unaffected | □ Limits buttons, writing or walking: call | □ Washing, dressing or walking difficult: urgent review | □ Sudden major weakness / paralysis: emergency help |
| **Muscle / joint pain** | □ None | □ Mild aches; normal activity | □ Pain limits tasks / sleep: call | □ Pain limits self-care: urgent review | □ Sudden severe deterioration: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Mouth soreness / ulcers** | □ None | □ Mild soreness; eating normally | □ Painful; soft food needed: call | □ Cannot eat / drink enough: urgent review | □ Major breathing difficulty: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

**Phase 2 · reusable record** · copy this sheet for another cycle of the same phase.

## Pembrolizumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |

## Doxorubicin · Anthracycline chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Mouth soreness / ulcers** | □ None | □ Mild soreness; eating normally | □ Painful; soft food needed: call | □ Cannot eat / drink enough: urgent review | □ Major breathing difficulty: 112/999 |
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

## Cyclophosphamide · Alkylating chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Urinary symptoms** | □ Usual pattern | □ Mild change: report | □ Pain or frequent urination: call | □ Cannot pass urine / marked bleeding: urgent review | □ Collapse or severe illness: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

**After surgery · pembrolizumab record** · copy this sheet for another cycle of the same phase.

## Pembrolizumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

**After surgery · additional record** · copy this sheet for another cycle of the same phase.

## Pembrolizumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

## Pembrolizumab 400 mg + carboplatin AUC 5 / paclitaxel → dose-dense AC

NCCP 00860 v2 · 21-day cycle

Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 5 on Day 1. Pembrolizumab 400 mg on Day 1 of cycles 1 and 3 only (every six weeks). Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 14 days. Pembrolizumab 400 mg on Day 1 of cycles 5 and 8 only; the six-week clock continues across the change in chemotherapy. G-CSF support is required for every dose-dense AC cycle; the team prescribes its product and timing separately. After surgery: pembrolizumab 400 mg every 42 days for five doses, beginning six weeks after cycle 8 Day 1, subject to recovery, surgery and the oncology plan. Progression preventing definitive surgery or unacceptable pembrolizumab toxicity changes this plan; postoperative treatment is not automatic.

| 1 | 2 | 3 | 4 | 5 | 6 |
|---|---|---|---|---|---|

**Your contact plan**

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
**Emergency: 112 / 999.**

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00860/ · no patient information.

Draft for review · SACTCheck v0.84.13

# Sources and further information

Pembrolizumab 400 mg + carboplatin AUC 5 / paclitaxel → dose-dense AC · National Cancer Control Programme (NCCP) 00860

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **National Cancer Control Programme (NCCP) 00860 v2 · official regimen**
   https://healthservice.hse.ie/documents/6452/860_v2_Pembro_Carbo_AUC_5_Pacli_80_DDAC.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
4. **ASCO · neoadjuvant breast therapy and response assessment**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC8274745/
5. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
6. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
7. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
8. **NCCP antiemetics v8, Table 1 pp4–5**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf